

Raymundo Salcedo M.D.
6640 Van Nuys Blvd, Suite 101, Van Nuys, CA 91405

IN REFERENCE	:	JUAREZ CEJA, JUAN LUIS
DATE OF BIRTH	:	07/25/1978
DATE OF INJURY	:	05/06/2024
DATE OF REPORT	:	10/01/2024

INITIAL CONSULTATION

CHIEF COMPLAINT

Mr. Juarez is a 46-year-old male who was referred to pain clinic with chief complaints of neck pain, low back pain and left and right Achilles heel pain.

HISTORY OF PRESENT ILLNESS

The patient reports he was involved in a bicycle versus motor vehicle accident, where he was the helmet wearing ride over bicycle in La Tuna Canyon Boulevard in Los Angeles, where intersected the 210 Freeway. A car hit him on the right side of his body at the thigh and ankle level. The patient flew off and landed on his left shoulder and left hip. The patient was taken by ambulance to Olive View Hospital Medical Center, where he received treatment immediately after the accident. The patient filed a police report. From the bicycle versus auto accident, the patient has developed neck pain, low back pain and left and right Achilles heel pain. The patient has now been referred to me for consultation and management of his pain.

PAST MEDICAL HISTORY

None.

The patient denies any similar bicycle accidents or similar injuries or symptoms.

PAST SURGICAL HISTORY

Left clavicle ORIF.

ALLERGIES

To penicillin.

CURRENT MEDICATIONS

None.

FAMILY HISTORY

Noncontributory.

PERSONAL HISTORY

Denies alcohol, recreational drug or cannabis use.

CURRENT SYMPTOMS : CERVICAL SPINE

The patient is right-hand dominant. He has pain in neck, which is frequent and severe. The pain is on the right side of his neck and radiates to the right shoulder and down the right arm. The patient reports numbness and tingling in the fingers of the right hand more than the left. The neck pain is associated with tension, tightness and stiffness. The pain radiates down the right hand. The patient feels numbness, tingling and weakness most in the right middle finger, but also in the left middle finger. The neck pain is associated with tension, tightness and stiffness.

The symptoms are increased by twisting, turning, extending the neck, flexing the neck, keeping the neck in awkward or prolonged position, activity involving the use of neck muscles, vigorous activity, cold environment, and bending over.

CURRENT SYMPTOMS : LUMBAR SPINE

The pain in lower lumbar area that is constant, moderate to severe. The pain is in the midline, radiates down the buttocks, down the legs to the Achilles heel, where the patient reports numbness, tingling and pain.

The patient avoids strenuous lifting, carrying, pulling, pushing, stooping, and bending because of back pain.

CURRENT SYMPTOMS : LEFT AND LEFT HEELS

There is pain in the left and right Achilles heel area next to the medial and lateral malleolus. The pain is exacerbated by stepping forward and pointing the toes up.

PHYSICAL EXAMINATION

The patient is 6 foot 2 inches, 109 pounds male, who moves around the examining room with slow and deliberate gait.

SPINAL EXAMINATION - CERVICAL SPINE

Palpation of the spine reveals tenderness (graded 0=none; 1+=mild; 2+=moderate; and 3+=marked) in the following regions:

	Right	Midline	Left
Cervical Region:	3+	1+	3+
Spinous Processes:	3+	1+	3+

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Paraspinous Muscles:	3+	1+	3+
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Trapezius Muscles:	3+	1+	3+
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RANGE OF MOTION - CERVICAL SPINE

Range of motion degrees are an average of three motions performed by the patient.

Cervical Spine Range of Motion	Degrees	Pain
Flexion:	15	+
Extension:	10	+
Right Bending:	15	+
Left Bending:	15	+
Right Rotation:	30	+
Left Rotation:	30	+

SENSATION TESTING – CERVICAL SPINE

Sensory testing to light touch and cold (by alcohol swab) is intact bilaterally symmetric in C5-T1 dermatomes.

MOTOR STRENGTH - UPPER EXTREMITIES

	Right	Left
Deltoids:	4/5	4/5
Rotator Cuffs:	4/5	4/5
Biceps:	4/5	4/5
Triceps:	4/5	4/5
Supinators:	4/5	4/5
Pronator Teres:	4/5	4/5
Wrist Extensors:	4/5	4/5
Wrist Flexors:	4/5	4/5
Hand Intrinsic:	4/5	4/5

REFLEXES - UPPER EXTREMITIES

	Right	Left
Biceps:	1+	1+
Triceps:	1+	1+
Brachioradialis:	1+	1+

SPECIAL TESTS - CERVICAL SPINE

	Left	Right	Expected
Cervical Compression Test	Positive	Positive	Negative
Cervical Facet Loading Maneuver	Positive	Positive	Negative
Cervical Facet Tenderness	Positive	Positive	Negative

SPINAL EXAMINATION - LUMBAR SPINE

Heel walking and toe walking are unable to perform. Loss of lumbar lordosis. Skin color and temperature are normal.

Palpation of the spine reveals tenderness (graded 0=none; 1+=mild; 2+=moderate; and 3+=marked) in the following regions:

	Right	Midline	Left
Paraspinal Muscle Tenderness:	3+	2+	3+

RANGE OF MOTION - LUMBAR SPINE

Range of motion degrees are an average of three motions performed by the patient.

Lumbar Spine Range of Motion	Degrees	Pain
Flexion:	45	+
Extension:	15	+
Right Bending:	20	+
Left Bending:	20	+
Right Rotation:	25	+
Left Rotation:	25	+

SENSATION TESTING – LUMBAR SPINE

Sensory testing to light touch and cold (by alcohol swab) is intact bilaterally symmetric in L2-S1 dermatomes.

MOTOR STRENGTH - LOWER EXTREMITIES

	Right	Left
Quadriceps:	4/5	4/5
Hamstrings:	4/5	4/5
Gastrocsoleus:	4/5	4/5
Tibialis Anterior:	4/5	4/5

Extensor Hallucis Longus:	4/5	4/5
Common Toe Extensors:	4/5	4/5
Iliopsoas:	4/5	4/5
Gluteus Medius:	4/5	4/5

REFLEXES - LOWER EXTREMITIES

	Right	Left
Knee Jerks:	1+	1+
Ankle Jerks:	1+	1+

SPECIAL TESTS - LUMBAR SPINE

	Right	Left	Expected
Straight Leg Raising Test	Positive	Positive	Negative
Lumbar Facet Loading Maneuver	Positive	Positive	Negative
Lumbar Facet Tenderness	Positive	Positive	Negative
Sacroiliac Tenderness	Negative	Negative	Negative

LEFT AND RIGHT ANKLE

The left and right Achilles heel the tender to palpation on the lateral aspect. There is reduced range of motion actively and passively secondary to pain.

DIAGNOSTIC STUDIES

There is a cervical spine MRI without contrast dated 08/30/2024 which shows at C3-4, a 2.5 mm disc bulge, at C4-5, a 2.0 mm disc bulge, at C5-6, a 3.8 mm disc bulge, and at C6-7, a 2.5 mm disc bulge.

INITIAL IMPRESSION

1. Lumbar radiculopathy.
2. Cervical radiculopathy.
3. Bilateral Achilles tendinitis.
4. Cervical traumatic spondylopathy, bilaterally.
5. Lumbar traumatic spondylopathy, bilaterally.

RECOMMENDATION

1. The patient would benefit from epidural injection with corticosteroid versus PRP bilaterally at C4-5, C5-6 and C6-7.
2. The patient would benefit from MRI without contrast of lumbar spine and left and right Achilles tendons to evaluate for traumatic injury.

3. The patient would benefit from continuing chiropractic care and physical therapy treatment.
4. The patient would benefit from home exercise program focusing on spine strengthening, range of motion, flexibility exercises.
5. The patient would benefit from nonsteroidal anti-inflammatory and muscle relaxant medications as needed for pain.
6. Follow up in 1 month.

DISCUSSION

We had the pleasure to evaluate Mr. Juarez for a Pain Management Consultation with respect to injuries sustained in the above captioned accident. I have reviewed all of my records and diagnostic studies available as summarized above, in conjunction with this evaluation.

The patient should continue with physiotherapy and chiropractic care. The vast majority of the symptoms are adequately managed and resolved with aggressive treatment directed at improving muscle strength and joint mobility. Treatment to prevent degeneration of the muscles and decrease onset of arthritis in the joints have prognostic implications for rehabilitation and functionality in injured patients. The treatments range from \$4,000 per month during the aggressive phase of treatment to \$1,000 per month for maintenance therapy.

In addition, the patient should do home exercise. The recurrent symptoms are adequately managed and resolved with exercise under improved muscle strength and joint mobility. Treatment to prevent degeneration of the muscles and decrease onset of arthritis in the joints have prognostic implications for rehabilitation and functionality in injured patients.

The patient would benefit from an MRI scan of the lumbar spine and left and right Achilles.

If the pathology is obvious on the MRI scan and is confirmed by a physical exam, which is consistent with the pathology, then treatment should be directed to the pathology, which is usually a herniated or degenerated disc. These may result in low back pain sometimes radiating along the course of the affected nerve or a combination of both. Treatment is by means of a transforaminal epidural steroid injection to relieve the symptoms by decreasing the nerve inflammation and alleviating the pain. These are performed under fluoroscopic guidance in a surgery center and cost approximately \$17,000 each, which includes the cost of the surgical facility and the anesthesiologist.

The patient would benefit from epidural injection with corticosteroid versus PRP bilaterally at C4-5, C5-6 and C6-7. These are performed under fluoroscopic guidance in a surgery center. Depending on the complexity of the procedure, the cost will be \$13,000 to \$17,000 each, which includes the cost of the surgical facility. Upon reevaluation of the patient after the first injection and based on their response and disease progression, recommendations for further procedures may be warranted.

We have discussed the risks and benefits of the procedure in detail as well as the costs of the procedure.

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The patient will return to my office in one month for follow-up examination.

DISCLOSURE NOTICE:

This report is for medical/legal assessment of injuries and is not to be construed as a complete physical examination for general health purposes. Only those symptoms which are believed to have been involved in the injury or that might relate to the injury have been assessed. The patient has the right to obtain an evaluation for general health purposes with a personal physician.

Very truly yours,

A handwritten signature in black ink, appearing to read 'Ray Salcedo', written over a horizontal line.

Raymundo Salcedo, M.D.

Pain Management Specialist

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